

Company Name Branding List all Team Members Tagline or Short statement of purpose of company	Bro2Bro - Company Name	1 Company
	Amir Houston - Project Manager, Researcher, Communications, Presentation	45 sec Required elements
	Bryan Smith - Pitch Team, Presentation, Communications	Company name: bro2bro
	David Adedeji - Tech Team (Backend)	Tagline: "Helping black health through hair"
	Durron Huggins - Pitch Team	All 7 team members listed by name: Amir Houston, Bryan Smith, David Adedeji, Durron Huggins, Kauri Soumanou, Lincoln Chitswa, Yahya Sridjajamerta
	Kauri Soumanou - Research, Presentation, Data Management	Brief role labels next to each name if possible (e.g. tech, research, business, design)
	Lincoln Chitswa - Tech Team (Frontend), Presentation	Visual suggestion
	Yahya Sridjajamerta - Project Manager, Pitch, Presentation	Logo or stylized "bro2bro" wordmark large and centered
		Tagline beneath it in a smaller, lighter weight
		Team names in a clean row or two-column list at the bottom

Helping black health through hair

Tell a story.

What is the human dimension of the problem and why should we care?

What data do you have to show the size of the problem?

Show data about whether the problem is getting better or worse without your product.

Add a little bit of body text

2
Problem

90 sec

Story to tell first (before any data)

One man, named, real or composite – e.g. "Marcus, 44, Pine Bluff. Father died at 53 of a heart attack. BP reading at his barbershop: 178/108. Left with no follow-up. Three months later – a minor stroke."

Single line: "The barbershop started the conversation. Nothing continued it."

Data on the size and direction of the problem

Black men have the lowest life expectancy of any major demographic group – 4.8-year gap vs. white men

60% of Black Arkansans have high blood pressure – half completely uncontrolled (UAMS PRC)

Black men die of hypertension at nearly 3× the rate of white men

43% more likely to seek care in the ER than through preventive settings

800,000 excess cardiovascular deaths nationally from 2000–2022 (JACC 2024)

UAMS Barbershop Talk reaches only 15 of 75 Arkansas counties – the rural gap is massive

Without intervention: this disparity is not improving – the life expectancy gap has been stagnant since the 1980s

The core problem statement

"The problem is not awareness – it's sustained engagement after the first conversation"

Citation on slide: Jordan, H. et al. (2024). American Journal of Men's Health, 18.

Who is the customer of your product?

Black men around 18-40 who are seeking info on mental/physical health through trusted people

Why do they need it, to make what impact?

to help the demographic’s health and reduce the amount of

How big is the market?

What are the segments of your market?

What is your customer value proposition and what was your customer validation process?

In other words, do you really know details about the people who need your product or are you guessing?

3

Customer

60 sec

Two distinct customers — show them separately

End user (pays nothing): Black men ages 18–50 in Arkansas, particularly those with limited healthcare access, uninsured or underinsured, in urban and rural settings

Paying customer (B2B): Health insurers (BCBS Arkansas), hospital systems (UAMS), Medicaid managed care organizations, employers with large Black male workforces

Why they need it

Men: trusted, non-clinical, always-available health support in a form they'll actually use

Payers: reducing 1 ER visit saves \$1,800+ — preventive tools pay for themselves at scale

Market size

22 million Black men in the US — total addressable market

~200,000 Black men in Arkansas — serviceable addressable market at launch

143,000 barbershops nationally (~3 barbers each = 429,000 potential health ambassadors)

Customer validation (be specific here)

Reference the UAMS Barbershop Talk program (Camille Hart, UAMS College of Public Health) as community validation anchor

Cite Jordan et al. 2024 — barriers to preventive care utilization among Black/African American men

If your team spoke with any Black men, barbers, or community members this week — name them or the number here

"Customer validation process" is an explicit scoring item. If you didn't speak to anyone yet, say "our validation is grounded in peer-reviewed research and the UAMS program data" — but if any team member spoke to even one person this week, that person belongs on this slide.

What is your solution?

Why is it better than current alternatives or the competition?

How does it work?

What, generally, does it do?

(More details of this in the demo.)

4
Solution
60 sec
What it is
bro2bro is a barber-integrated mental and physical health platform
Extends the trusted barbershop conversation into sustained digital engagement
Works via SMS — no app, no smartphone required
How it works (three steps — make this visual on the slide)
Step 1 — In the chair: Barber shows QR code or tablet. Man answers a 90-second health check-in (blood pressure, stress, sleep)
Step 2 — The text: AI companion starts a text conversation using motivational interviewing — language that feels like a friend, not a clinic. Connects him to local resources
Step 3 — Accountability: Weekly check-ins via text. Optional "Trustee" (trusted friend or family member). Peer mentor matching. Barber tracks referrals via dashboard
Why it's better than alternatives
Generic apps (BetterHelp, Teladoc): not built for Black men, require smartphones, no community trust layer
Barbershop Talk alone: no digital follow-up, covers 15/75 Arkansas counties, no mental health component
bro2bro: culturally specific, SMS-first (any phone), community-embedded, privacy-first, multi-touchpoint
BARBER-1 trial evidence: barbershop + professional follow-up = 63.6% BP control vs. 11.7% without — we are the scalable follow-up layer

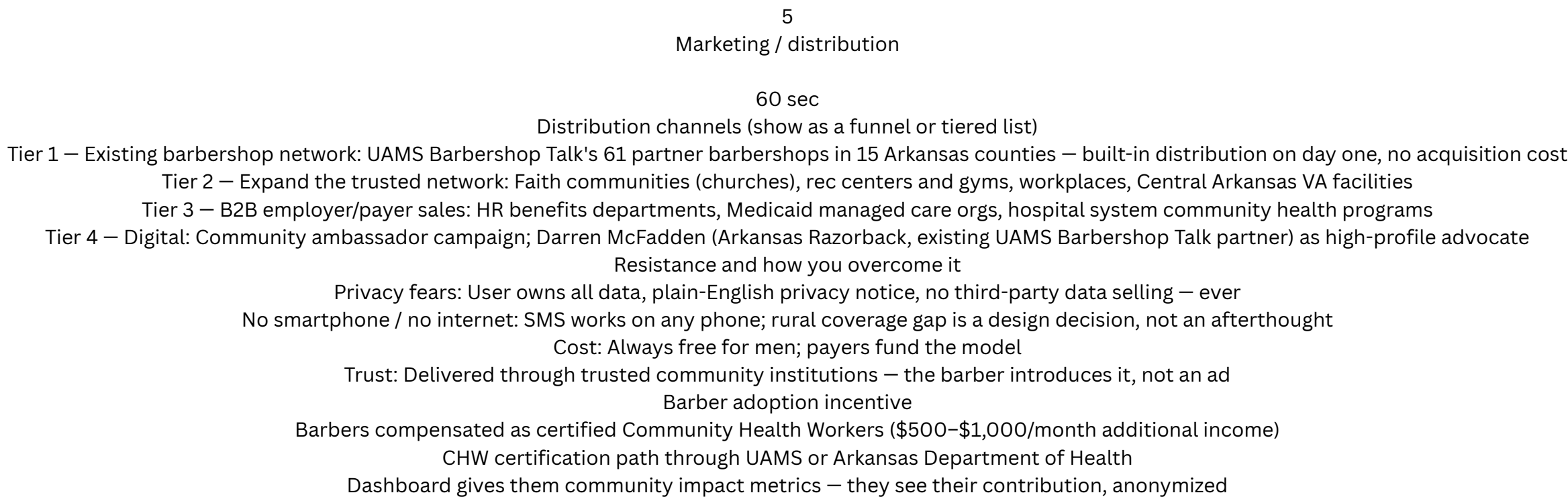
How will customers get your product?

What resistance is there and how will you overcome it?

What must happen for people who need your product to access it?

Can they afford it?

How will you reach out to more people who need your product?



How are you addressing regulatory compliance, cybersecurity and privacy issues with your product?

How will you address the challenges in the healthcare ecosystem and the market?

Show that you really understand what the drivers are in the market.

What steps are needed to enter the market and grow?

6

Healthcare & regulations

60 sec

Regulatory compliance (three clear columns work well visually)

Wellness tool, not a diagnostic: AI says "your reading is worth discussing with a doctor" — never "you have hypertension." Keeps us out of FDA medical device regulation at launch

HIPAA-compliant architecture: Business Associate Agreements with any health system partners; encrypted data storage; audit logging

42 CFR Part 2: Mental health and substance use data stored in a separately encrypted partition — protected beyond standard HIPAA, never shared without explicit consent

Crisis escalation: 988 Suicide & Crisis Lifeline surfaced automatically on crisis language detection — no delays, no questions first

Privacy architecture (key trust differentiator)

Anonymous user IDs — no real names stored

Zip codes: processed in memory only, never stored, never transmitted to third parties

No third-party ad trackers — no Facebook Pixel, no Google Analytics

User-controlled data deletion: users can delete everything, receive SMS confirmation within 24 hours

Learned from BetterHelp's \$7.8M FTC fine — our technical architecture matches our privacy promises

Market entry steps

Launch as wellness tool → IRB partnership with UAMS for research validation → BCBS Arkansas pilot contract → Arkansas Medicaid MCO contracts → National scale in Southern states

Explain how you and your teammates learned about this challenge.

What does the team think it will take to make the product financially viable/profitable or sustainable through some funding mechanism.

What people, skills and/or agents do you need to make it happen?

Does anyone on your team want to make this business happen?

If so, why? If not, why not?

7

Building a business

45 sec

How the team learned about this challenge

Brief, genuine — 1–2 sentences. How did you encounter Challenge 07 and what made you choose it?

If any team member has a personal connection to this health disparity — this is the moment to say so briefly

Revenue model

B2B2C — men use free, payers pay per member per month (\$10–\$25 PMPM)

10,000 members at \$15 PMPM = \$1.8M ARR from one insurer contract
Additional revenue: federal grants (CDC, HRSA), Medicaid CHW billing (\$25–\$50/encounter), healthcare provider directory subscriptions
Year 1 target: \$500K–\$1M (pilot phase); Year 3: \$10M–\$50M (regional scale)

Funding path

CDC Prevention Center grants + HRSA community health grants (UAMS has existing relationships)

BCBS Arkansas pilot contract as first commercial revenue

Jumpstart Nova — \$55M fund specifically for Black-led healthcare companies, backed by AHA & HCA Healthcare

High-profile advocates: Darren McFadden (existing UAMS partner), Magic Johnson Enterprises (healthcare track record)

Team intent — each person answers personally

One genuine sentence per team member about why they'd build this. Not scripted. Practice it so it's real.

Judges explicitly score "does anyone on the team want to make this happen?" — this is worth real points. Don't skip the personal statements. Even "I want to build this because my uncle died of a preventable heart attack" is worth more than any polished financial projection.

Specific Questions for Each Judge in Notes from slides 8-13

Alicia Berkemeyer — Arkansas Blue Cross Blue Shield
Insurance executive at Arkansas's dominant health insurer

Her lens: cost containment, actuarial evidence, member engagement, HEDIS metrics, contract structure, data ownership, ROI proof

"You cite the BARBER-1 study for your 63.6% figure — but that's not your data. Where's YOUR evidence that bro2bro specifically reduces costs?" gap

"You're right — we don't have outcomes data yet because we're five days old. What we have is mechanistic evidence: the BARBER-1 trial proves the intervention model works when there is professional follow-up. Our platform IS that follow-up layer, using the same motivational interviewing techniques. What we're asking BCBS to fund is a 6-month pilot to generate our own evidence. We'd define endpoints together — BP control rate, ER utilization change, preventive care visit completion — and build a payment structure contingent on hitting agreed benchmarks. We don't need you to take our word for it. We need you to help us prove it."

"How do we structure the contract? What outcomes are you committing to and how do you measure them?" standard

"We propose a pilot contract: BCBS enrolls a defined cohort of high-risk Black male members — those with a hypertension diagnosis and no recent preventive care visit. We track: engagement rate at 30/60/90 days, BP readings submitted, preventive care referrals completed, and ER utilization for the cohort vs. a control group. At 6 months we review together. If the data supports it, you expand to a larger population on a per-member-per-month contract. If not, the pilot contract ends. We carry the risk; you hold the evaluation criteria."

"What HEDIS measures does this move and why should that matter to us?" standard

"Directly: Controlling High Blood Pressure (CBP) — one of the most heavily weighted HEDIS measures for commercial and Medicaid plans. Also: Colorectal Cancer Screening (COL), if we add those reminders to the platform, and Initiation and Engagement of Substance Use Disorder Treatment (IET), given our alcohol screening component. HEDIS performance directly affects your Star Ratings, which affect your CMS reimbursement rates. A one-point improvement in BP control on a population this size moves real money."

"Who owns the health data collected from our members?" gap

"The member owns their data — full stop. bro2bro serves as a Business Associate under HIPAA, with a signed BAA with BCBS. We never sell member data. Aggregate, de-identified population analytics — for example, 'elevated stress rates among members in county X correlate with reduced preventive care utilization' — are available to BCBS as a population health insight product, with member consent built into the onboarding flow. Individual member data never passes to BCBS without that member's explicit, revocable permission."

"What's the projected enrollment rate and how did you arrive at it?" gap

"For the pilot, we'd target 1,000 members. Enrollment through the UAMS barbershop network — 61 shops currently, each seeing 40–60 clients weekly. Conservative: 5% of barber clients opt in after being introduced by their barber. That's 120–180 enrollments per week across the network. We reach 1,000 in roughly 6–8 weeks. Importantly, this enrollment happens through a trusted messenger — the barber — which research shows produces significantly higher uptake than cold app installs or insurer mailers. We're not estimating from an App Store; we're estimating from an existing trusted relationship network."

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Arkansas Medicaid CHW Reimbursement Status
The Legislation: Arkansas passed Act 435 (the Community Health Worker Certification Act) and Act 140 (the Healthy Moms, Healthy Babies Act). These laws officially established a statewide CHW certification program overseen by the Arkansas Department of Health (ADH) and created a legal mandate for CHW services to be reimbursed by Arkansas Medicaid and private insurers.
The Current Operational Status: While the legal pathway is authorized, full operational implementation is still in the rulemaking and federal approval phase. The state is working on the exact Medicaid State Plan Amendments (SPAs) and billing codes. Furthermore, reimbursement will be strictly tied to state-certified CHWs.
Because the direct Medicaid billing pathway is not yet fully active for general community health functions, you cannot use it as an immediate source of funding for the barbers.
Revised Framing for the Barber CHW Model
To keep the model viable right now, frame the \$500–\$1,000/month compensation as an initial B2B structure that transitions into a Medicaid-reimbursed model later.
Phase 1: The B2B / Value-Based Contract (Immediate)
The Source: The monthly stipend is paid directly to the barbershop via a Business-to-Business (B2B) contract with a hospital system, Accountable Care Organization (ACO), or a public health grant.
The Justification: The healthcare partner funds this out of their marketing, community benefit, or value-based care outreach budgets. The barbers act as trusted liaisons who drive patient enrollment, improve preventative care compliance (e.g., hypertension screenings), and reduce costly ER visits.
The health system recaptures the investment through improved quality metrics and patient acquisition.
Phase 2: Medicaid & Insurance Reimbursement (Future-Proofing)
The Source: Direct billing through Arkansas Medicaid and private health plans.
The Justification: As the Arkansas Department of Health finalizes the administrative rules for Act 435, the program will sponsor the participating barbers to go through the official state CHW certification training. Once the barbers are state-certified and the DHS billing codes go live, the partnering healthcare system will be able to bill Medicaid directly for the screenings and navigation services performed at the shop, making the stipend self-sustaining.
Proposal Tip: Frame the current lack of an active Medicaid billing pipeline as an advantage. By launching under a B2B contract now, your program will have an established network of trained barbers ready to be certified the moment Arkansas opens up direct Medicaid billing.

Commercializes UAMS research; knows what makes health startups investable vs. grant-dependent

His lens: IP, commercialization path, research validation, clinical evidence, investment readiness, academic-industry partnerships

"Are you actually partnered with UAMS Barbershop Talk, or are you just citing their data?" gap

"We are students building on UAMS's published research and the publicly available data from Barbershop Talk. We do not have a formal partnership yet — that's honest. What we're proposing is that this hackathon becomes the foundation for that conversation. UAMS BioVentures is literally in this room. Camille Hart designed the challenge. What we're saying is: the technology to extend your program exists, we can build it, and the right next step is a formal pilot agreement where bro2bro integrates into the Barbershop Talk workflow and UAMS provides the community relationships and IRB oversight. That's the partnership we want."

"What's your IP? Can a bigger company just clone this tomorrow?" gap

"Our competitive moat is not a patent — it's the network. Every barber we certify as a Community Health Ambassador, every UAMS barbershop we integrate into, every peer mentor we onboard, every BCBS member we enroll — those relationships are not copyable. The larger a competitor is, the harder it is for them to build trust in these specific communities. Teladoc has \$18 billion and they cannot walk into a Pine Bluff barbershop and have the conversation that a trusted barber can have. Our IP is the network, the training system, and the culturally specific conversation design — and those take years to replicate."

"What does the path from hackathon to commercially viable actually look like — specifically?" standard

"Month 1–3: UAMS IRB-approved pilot with 10 barbershops, 100 men, SMS-only MVP. Collect engagement data and preliminary BP readings. Month 3–6: Apply for CDC Prevention Center funding through UAMS — they already have the grant infrastructure. Months 6–12: Present pilot data to BCBS Arkansas for a paid contract conversation. Year 2: Jumpstart Nova Series A application — \$55M fund, AHA and HCA Healthcare backed. Year 3: Regional expansion into Mississippi, Louisiana, and Georgia with payer contracts in each state. The IRB data is the unlock for everything else — that's why UAMS partnership is our first move, not our last."

"Why won't UAMS just build this themselves?" gap

"UAMS is a research institution and healthcare provider — not a technology startup. Their competitive advantage is clinical expertise, community trust, and research infrastructure. Our advantage is software velocity, commercial model design, and the ability to move at startup speed. UAMS BioVentures exists specifically because UAMS recognizes that the fastest path to commercializing their research is through external companies, not internal development. We're not competing with UAMS — we're asking to be the commercial vehicle for research they've already validated."

Erin Parker — Arkansas Children's

Executive/leader at Arkansas Children's; likely clinical or program implementation lens

Her lens: clinical handoffs, implementation feasibility, accountability, evidence base, community impact measurement

"What happens when the AI conversation reaches a point where someone actually needs a clinician? What's the handoff?" gap

"This is the most important transition in the whole platform and we've designed it carefully. When the AI detects a risk flag — language suggesting depression, high-risk BP readings, substance use concerns — it immediately surfaces two things: the appropriate crisis or clinical resource, and an offer to schedule an appointment directly. If the man consents, the platform sends a booking request to a partnered clinic through a FHIR API — no phone call required on his end. For mental health, we maintain a directory of licensed Black therapists and culturally competent providers in Arkansas, updated quarterly. The AI's job is to get him to say yes. Our job is to make the yes as frictionless as possible."

"Who is responsible if someone in crisis reaches out and the AI misses it?" gap

"We carry platform liability insurance — standard for wellness applications in this space. More importantly, our crisis protocol is automated and human-backed: keyword detection triggers 988 immediately, a human reviewer is alerted within minutes. The AI is not the last line of defense — it's the first responder. A human is always in the loop for any crisis flag. Our terms of service are explicit that bro2bro is a wellness tool, not a clinical service, which limits but does not eliminate liability. We've structured the crisis escalation so that we cannot be accused of gating access to help — it surfaces unconditionally."

"Is there any evidence that AI-based motivational interviewing works — not just human MI?" standard

"Yes — a 2025 systematic scoping review published in JMIR found that AI systems delivering MI-based conversations are 'generally feasible, usable, and positively received,' with moderate to strong alignment with MI principles in roughly 40% of studies reviewed. That's early-stage evidence, not gold standard — but it's meaningful, and this space is moving quickly. Critically, the studies show highest effectiveness when the AI is delivered through a trusted introduction, which is exactly our model: the barber introduces bro2bro, giving the AI credibility that a cold app install cannot."

Salena Wright-Brown — Central Arkansas Veterans Healthcare System

Senior leader at the Central Arkansas VA; works with veterans' complex physical and mental health needs

Her lens: Black veterans, PTSD/trauma, federal procurement, VA system integration, rural veteran access, Section 508, suicide prevention

"How does this serve Black male veterans specifically? They're at disproportionate risk for suicide and substance use." standard

"Black veterans are one of the highest-need segments within our target population. They face the compound of racial health disparities AND military trauma — PTSD, TBI, MST — at rates not reflected in most digital health tools. For veterans, we'd differentiate the crisis escalation: 988 has a Veterans Crisis Line option (press 1), and we'd surface that specifically for users who identify as veterans. The peer mentor program is especially powerful here — there is already a peer support specialist model in the VA system, and veteran peer mentors carry unique credibility with other veterans that no clinician can match. A partnership with Central Arkansas VAHCS to co-develop the veteran track of bro2bro would be a natural extension."

"The VA has existing mental health and community health programs. How does bro2bro integrate rather than compete?" standard

"We don't replace VA services — we serve as a trusted on-ramp to them. Many veterans who need care resist walking into a VA facility because of stigma or past negative experiences. bro2bro reaches them where they are — in the barbershop, via text — and, when they're ready, navigates them directly to VA services. Our platform could literally include VA appointment scheduling, VA mental health resource links, and VA peer support contact information. We're a referral engine for existing VA programs, not a competing clinical service."

"How would you actually work with the VA? Federal procurement is complex and slow." gap

"We're realistic about federal procurement timelines — a direct VA contract is a Year 3 target, not a Year 1 path. The near-term collaboration is informal: we include VA resources in our care navigation, we invite VA community health workers to use the barber dashboard, and we invite VA facilities in Arkansas to become bro2bro access points. A formal procurement path would likely run through the VA's Veterans Health Innovation (VHI) or ATLAS program, which has an existing telehealth and community partnership track. We'd pursue that with UAMS as a co-applicant."

"Does your platform meet Section 508 accessibility standards?" gap

"SMS is inherently accessible — it doesn't rely on visual interfaces, works with screen readers and assistive technology on any phone, and functions without internet. For any app or web interface we develop, Section 508 compliance is a development requirement, not an afterthought. We'd include it in our technical specification from the first line of code. This is especially important for veteran users, where sensory and cognitive accessibility needs are common."

Tech judges — AbuHalimeh, Berleant, Wu (UA Little Rock)

Score the demo (40%) but may ask pitch questions during Q&A

Wu's specific research: LLM data lifecycle risk detection and AI governance — she will probe this hard

"What's your LLM data lifecycle? What enters the model's context, what gets retained, and how do you ensure no PHI leaks into model logs?" gap

"This is a core governance question we've designed for explicitly. User messages enter the model context in real time and are not retained in model weights — we're calling a commercial LLM API, not training a custom model. Conversation history is stored encrypted in our database under anonymous user IDs, accessible only for the next conversation with that user. Logs from the LLM provider API calls are purged after 30 days. PHI is never sent to the LLM in raw form — we pass only wellness context and conversation content, never names, phone numbers, or identifiable health record data. The anonymous ID is the only identifier that touches the AI layer."

"How do you audit the AI for racial bias in its responses? An AI that gives different quality advice to Black users than white users would be harmful." gap

"This is a real risk and we take it seriously. We have two controls. First, our system prompt is deliberately designed for this population — it doesn't have to infer cultural context, it's built in. Second, we conduct response auditing: a community advisory board of Black men and Black healthcare professionals reviews a sample of AI outputs quarterly, specifically testing for language that sounds clinical, dismissive, or culturally inappropriate. We also test adversarially — we run the same health scenarios through the AI and review whether responses differ in quality by inferred user background. We can't eliminate bias entirely, but we can detect and correct it systematically."

"How does the AI improve over time without retaining sensitive user data?" standard

"We use aggregate, de-identified conversation analytics — not individual user data — to understand what conversational patterns lead to positive outcomes like care referrals or BP readings shared. We also work with UAMS researchers who can conduct IRB-approved studies using properly consented data to generate improvement signals. The model itself is improved by refining the system prompt and conversation guidelines based on these aggregate insights, not by retraining on user conversations. Privacy and performance are not in conflict if you design the feedback loop correctly from the start."

Gaps in our Pitch that we need to look at

No original validation data

Your evidence is entirely borrowed from BARBER-1 and UAMS. A judge will say "that's not your data." You need to frame this honestly: you're building the follow-up layer that BARBER-1 proves is needed, and your pilot will generate your own evidence.

Fix: Preempt it in Slide 3 — "We're building on proven research, and here's how we'll generate bro2bro-specific outcomes data."

Barber CHW compensation is state-specific and not confirmed for Arkansas Medicaid CHW billing varies by state. You're claiming \$500–\$1,000/month for barbers as CHWs. If Arkansas Medicaid doesn't have a CHW billing pathway, this falls apart.

Fix: Research tonight whether Arkansas Medicaid reimburses CHW services (check Arkansas DHS / Medicaid website). If yes, cite it. If not, frame compensation as coming from the B2B contract, not Medicaid billing, until the pathway is established.

Answered on 9

The Trustee feature has a privacy risk

A judge (especially the BCBS or VA judge) might ask: what if a Trustee is an abusive partner, an employer, or someone who could use the health information against the man? This is a real scenario.

Fix: Add one line in Slide 6: "Trustees see only what the user explicitly chooses to share. There is no automatic disclosure. The feature is entirely opt-in and revocable."

UAMS partnership is aspirational, not confirmed

If a judge (especially Eric Peterson from UAMS BioVentures) asks "are you partnered with UAMS?" and you say no, it undercuts everything. Saying you're "building on" their research is fine but needs to be framed carefully.

Fix: Frame it as "we are building the commercial vehicle for their research, and the next step is a formal partnership conversation — one we're hoping to start in this room."

The clinical handoff mechanism is vague

You say the platform "connects men to care" but the specific mechanism — how an at-risk man goes from a text conversation to an actual appointment — is not spelled out in the pitch.

Fix: Be specific in Slide 4. "When a risk flag is detected, the platform surfaces two local providers and an SMS booking link. If he texts 'book,' it sends a scheduling request directly to that clinic's system."

LLM data lifecycle — Ningning Wu will ask this

Wu's research is specifically on LLM data lifecycle risk detection. She will ask what happens to conversation data, whether PHI enters the model context, and how you govern the AI. If your tech team hasn't addressed this architecturally, it will show.

Fix: Brief the pitch team tonight on the specific answers (see Wu's Q&A above). If you don't know your AI's data retention policies, look them up — every major LLM API publishes them.

Peer mentor liability and quality control

You mention peer mentors but don't address how they're screened, trained, or liability-protected. A clinical judge will ask what happens if a peer mentor gives harmful advice.

Fix: Add one line in Slide 4: "Peer mentors share personal experience — they do not give medical advice. They are screened, trained in scope boundaries, and covered under the platform's terms of service, which explicitly distinguishes peer sharing from clinical guidance."

The tagline may undercut your multi-touchpoint story

"Helping black health through hair" implies barbershop-only. If a judge asks "what about Black men who don't go to barbershops?" — rural men, older men, young men who cut their own hair — your tagline works against you.

Fix: Either update the tagline ("...through community") or address it explicitly in Slide 4: "The barbershop is our launch channel — but bro2bro works through any trusted community setting, and via SMS for men who can't access any of them."